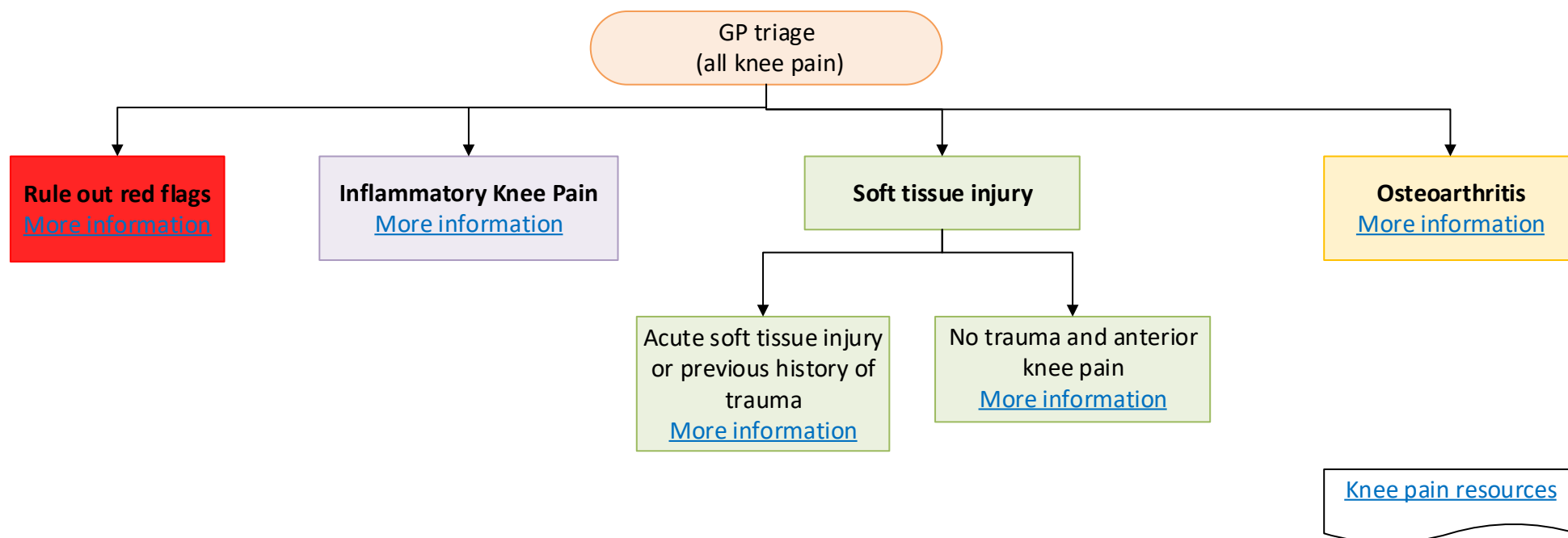


City and Hackney MSK knee pathway



Red flags for serious pathology – Knee Pain

NICE guidelines July 2017: Knee pain – assessment

What are the red flags for more serious pathology?

Red flags (or alarm features) are symptoms, signs, or risk factors that may indicate more serious pathology and warrant urgent further investigation, or immediate or urgent referral.

In a person with knee pain who has no history of trauma:

- **Red flags for infection** (septic arthritis or osteomyelitis) Redness, swelling, heat, and reduced movement of the knee, especially if: significant swelling appeared acutely, over less than 24 hours. Only one joint is affected, although in up to a fifth of people with septic arthritis, more than one joint is affected. Knee pain is severe or, in people with pre-existing joint disease (for example rheumatoid arthritis and osteoarthritis), out of proportion to the usual symptoms. Fever is present (although the absence of fever does not exclude septic arthritis); there is nausea, vomiting, or anorexia; or the person is otherwise systematically unwell (common in children). There are risk factors for infection: recent knee surgery (in particular, knee replacement), rheumatoid arthritis, intravenous drug use, immunosuppression (for example diabetes, use of long-term corticosteroids, alcoholism), or adjacent skin infection or ulceration.
- **Red flags for a tumor** (bone tumor, soft-tissue sarcoma, metastases, haematological cancer, or neuroblastoma) Increasing, persistent, or unexplained bone pain, tenderness, or swelling, especially if it is not in the knee joint itself but adjacent to the knee. Pain at night or at rest. Unexplained weight loss. Previous cancer. Sudden onset of pain (may indicate a pathological fracture, but can also occur in osteonecrosis). In adults, any palpable lump that is *any* of the following: larger than about 5 cm in diameter; deep to fascia; fixed or immobile; painful; or increasing in size (possible soft tissue sarcoma). In children, any unexplained mass that is *any* of the following: deep to the fascia; non-tender; progressively enlarging; associated with a regional lymph node that is enlarging; larger than 2 cm in diameter; or a recurrence after previous excision (possible soft tissue sarcoma).
- **Red flags for inflammatory polyarthritis** — Persistent synovitis, indicated by: Pain that is worse at rest or during periods of inactivity. Joint swelling, tenderness, and warmth — giving a 'boggy' feel on palpation. Stiffness in the morning and after inactivity that lasts more than 30 minutes (may be absent or last less than 30 minutes in children with juvenile idiopathic arthritis). Synovitis affecting other joints — symmetrical synovitis of the small joints of the hands and feet is typical in rheumatoid arthritis. A history of psoriasis, inflammatory bowel disease, or iritis (uveitis).

Please note: This list is not exhaustive

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Inflammatory Knee Pain

Patient presents with:

1. Pain/ swelling/ warmth/ morning stiffness
2. No trauma
3. Associated back pain/ polyarthralgia
4. GI/ GU symptoms
5. Psoriasis
6. Uveitis/at risk of sexually transmitted disease / recent upper respiratory tract infection



Possible investigations:

1. Bloods: CRP/urate/infection screen
2. Microscopy of joint aspirate
3. X-ray



Refer to Rheumatology via e-RS

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Acute soft tissue injury

NB: GP management:
PRICE & Analgesia
(not NSAIDs in first
48 hours)

Trauma with presence of one or more of:

1. True instability
 2. True locking
 3. True giving way
 4. Haemarthrosis
 5. Ligament injury
 6. Unable to weight bear
 7. Age under 40 with presence of effusion
- X-ray if # suspected, consider referral to A&E

Yes

No

(Consider MRI at
Homerton if under 40)
Weight bearing X-Ray; AP,
lateral and skyline

Immediate referral
to Homerton Sports and
Musculoskeletal Medicine
Clinic, via eRS
[Further information](#)

If athlete

Locomotor ESP referrals

Routine ESP appointments are on e-RS, please
use: Extended Scope (ESP) Assessment Service
- St Leonards- RQX20, which is indirectly
bookable: [Referral information](#)

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Shapes for video links:

Assess

1. Ligament stability

ACL: [Lachmans](#)

[Anterior draw](#)

PCL: [Posterior draw](#)

LCL: [LCL stress test](#)

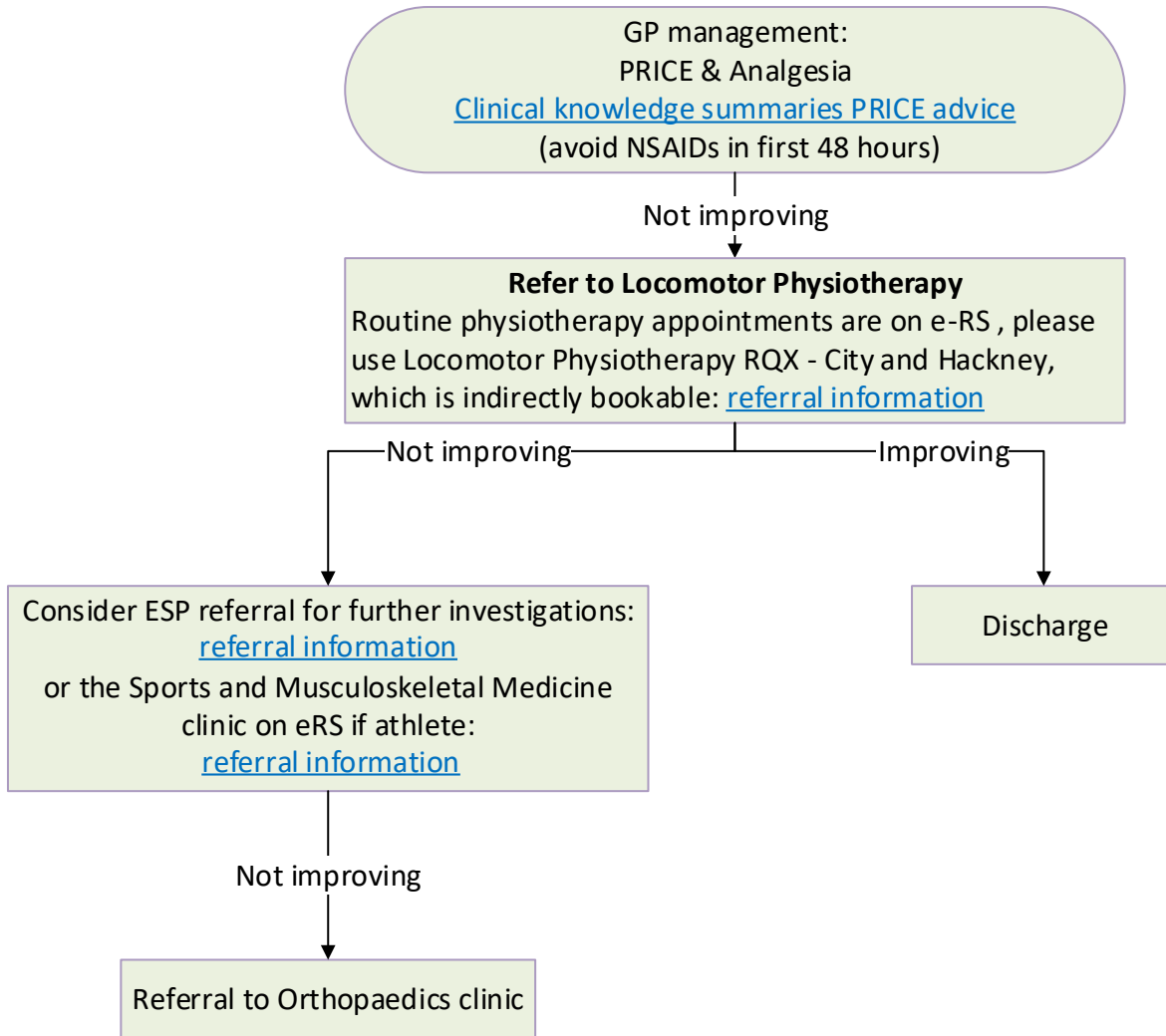
MCL: [MCL stress test](#)

2. Meniscus

[Joint line palpation](#)

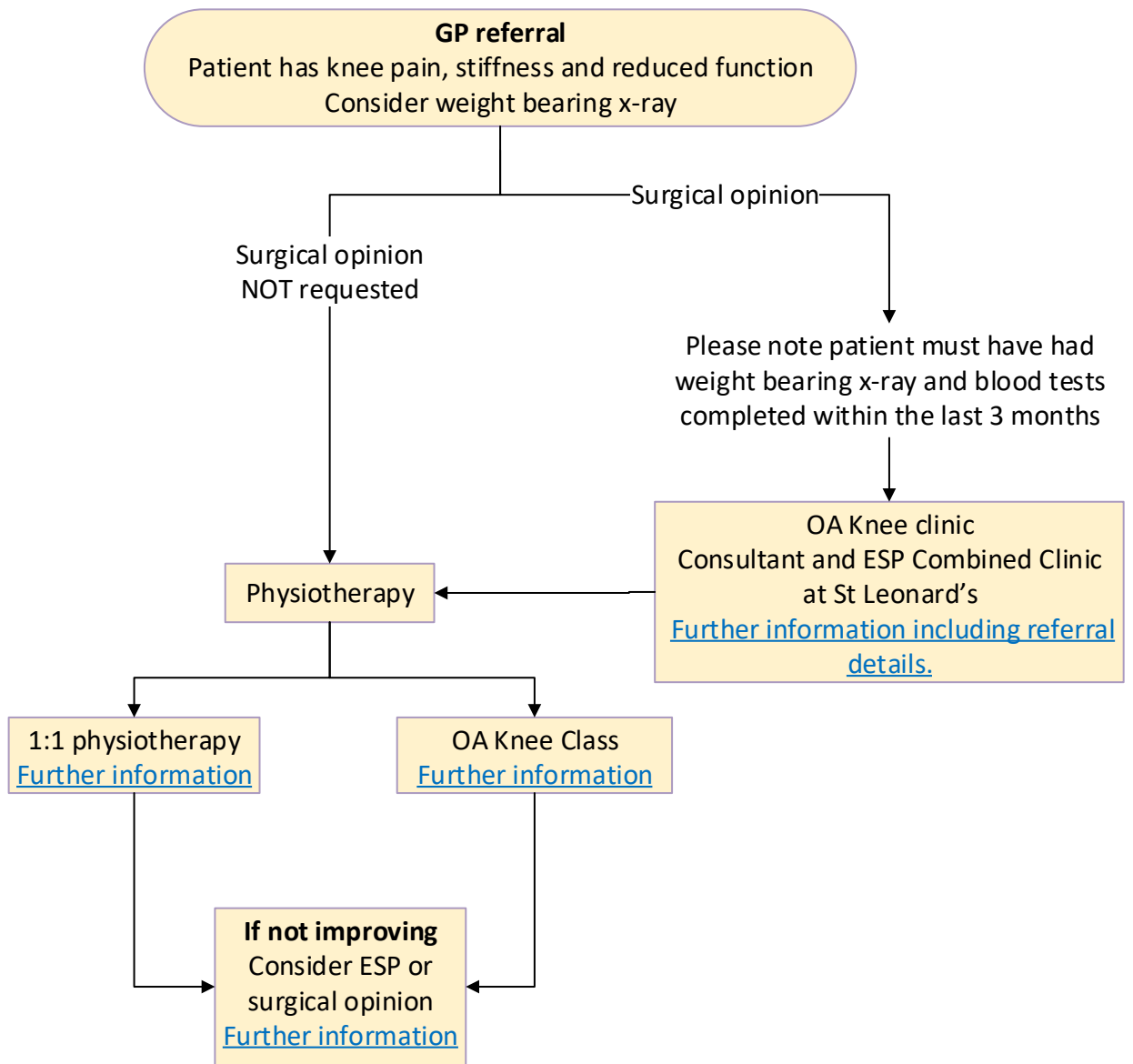
[McMurrays](#)

No trauma



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Osteoarthritis



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Locomotor Physiotherapy referrals

Routine physiotherapy appointments are on e-RS, please use Locomotor Physiotherapy RQX - City and Hackney, which is indirectly bookable.

e-RS Locomotor Advice and Guidance is available pre-referral.

Process for booking a referral into Locomotor Physiotherapy:

- Give the patient an appointment request form with UBRN number
- Please attach the patient referral in the normal way
- Advise the patient to phone CAS after 5 working days (Tel: 0207 683 4676). Only one phone call will be required to book an appointment

Note

- Urgent referrals should be attached and sent to locomotor@nhs.net.
- Transport for the first appointment will need to be organised by referrer.
- For queries please contact Hilda Walsh, Head of Locomotor Service, Tel: 0207 683 4378 or Elizabeth Slee or Kate Dean, Deputy Heads, Tel: 020 7683 4553).

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Locomotor ESP referrals

Routine ESP appointments are on e-RS , please use: Extended Scope (ESP) Assessment Service - St Leonards - RQX20, which is indirectly bookable.

e-RS Locomotor Advice and Guidance is available pre-referral.

Process for booking a referral into ESP assessment:

- Give the patient an appointment request form with UBRN number
- Please attach the patient referral in the normal way
- Advise the patient to phone CAS after to activate the referral
- Patient then phones back after 5 working days (Tel: 0207 683 4676)

Note

- Urgent referrals should be attached and sent to locomotor@nhs.net.
- Transport for the first appointment will need to be organised by referrer.
- For queries please contact Hilda Walsh, Head of Locomotor Service, Tel: 0207 683 4378 or Elizabeth Slee or Kate Dean, Deputy Heads, Tel: 020 7683 4553).

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OA Knee clinic
Consultant and ESP Combined Clinic at St Leonard's

This Wednesday pm clinic will be run at St Leonard's. Patients can be referred in by a GP, or following a course of treatment in the Locomotor Service.

The addition of this clinic brings a surgical opinion earlier into the pathway. This opinion is from an experienced consultant orthopaedic surgeon in conjunction with an Extended Scope Physiotherapist (ESP) and the patient. It reduces the number of steps in the pathway for the patient.

There are no diagnostics on site, therefore GP referrals need to come with a **weight-bearing knee x-ray** to facilitate fast decision-making. Patients who have come through the Locomotor Service will be electronically reviewed by an ESP prior to this appointment and appropriate radiography will have been requested. It is also essential that the patient has recent blood tests (in the last 3 months) - in particular to assess pre-operative haemoglobin levels, which can affect outcome of surgery.

Interventions include:

1. Pain control optimisation
2. Injection therapy
3. Joint clinical reasoning sessions
4. Shared decision making with the patient, ESP and Consultant
5. Patient referred for 1:1 physiotherapy

From this clinic patients can be listed for surgery. At the same time they will be given an appointment for pre-admission clinic and for 'Joint School' - a multi-disciplinary education session for patients due to undergo joint arthroplasty surgery.

If the patient has co-morbidities, or if it is identified that there may be reasons for a complex discharge, the patient can be referred to a pre-operative multi-professional clinic led by a Geriatrician (see box CGA Clinic).

Consider referral to physiotherapy in the first instance if the patient has not had physiotherapy before.

Referral

e-RS (Directly Bookable) or via the ESPs with recently blood tests showing Hb levels, and a weight bearing knee x-ray.

OA Intro Session Locomotor Service, St Leonards

- 1 hour 15 minutes education session
- Focus on holistic impact of OA/ Pain on persons life and discuss self management strategies
- Meeting NICE guideline recommendations
- Aim to facilitate behavioural change - particularly with exercise, weight loss & pacing
- Multiple topics covered as below
- Exercise
- Weight Loss
- Pacing
- Flare ups
- Goal Setting
- Sleep
- Pain
- Pharmacological and Surgical management

3 Week opt in Exercise Class

- Circuit Based Physiotherapy led exercise class
- Lower limb strengthening
- General aerobic fitness
- Building Self efficacy
- Patients have the option of repeating the 3 week class to get 6 weeks of exercise

All patients receive Signposting - Exercise and information

- Supporting patients to continue exercise, weight loss and self management independently
- Healthier together Hackney
- 12 Week exercise program
- Weight Loss Program also available
- New Age Games - Over 50s Exercise program
- One Pound One You Classes - £1 classes across the borough
- Walking Groups
- Hackney Council website provides up to date information
- Arthritis Research UK booklets provided

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Referral to Sports and Musculoskeletal Medicine Clinic

GP should refer to the Homerton Sports and Musculoskeletal Medicine clinic via The Electronic Referral Service (e-RS). Services are provided at 2 sites: Homerton Hospital and The Sir Ludwig Guttman Health and Wellbeing Centre (SLG)

The Sir Ludwig Guttman Health and Wellbeing Centre
40 Liberty Bridge Road
East Village Olympic Park
Stratford E20 1AS

Homerton Hospital Sports and Musculoskeletal Medicine Clinic, Physiotherapy department, Homerton University Hospital,
Homerton Row, London
E9 6SR
Tel: 020 8510 7835

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1:1 physiotherapy

One to one physiotherapy sessions as per the [NICE Osteoarthritis knee guidelines](#) including:

1. Exercise (local muscle and aerobic exercise)
2. Advice
3. Manual therapy

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Knee pain clinical resources

- NICE guidelines Overview of knee assessment: [click here](#)
- Chartered Society of Physiotherapy information for GP's page: [click here](#)
- NICE Clinical Knowledge Summary Knee pain – no history of trauma: [click here](#)
- NICE Clinical Knowledge Summary Knee pain – with history of trauma: [click here](#)
- British Journal of Sports Medicine – knee examination video's: [click here](#)
- NICE osteoarthritis clinical knowledge summary: [click here](#)
- Management of Osteoarthritis NICE flow chart: [click here](#)

Patient exercise leaflets and videos

- Knee pain exercises (Chartered Society of Physiotherapy): [click here](#)
- Knee exercises video (Chartered Society of Physiotherapy): [click here](#)
- Ageing well information (Chartered Society of Physiotherapy): [click here](#)
- Talking about your generation – Staying active leaflet: [click here](#)
- Physiotherapy advice and exercises for knee pain: [click here](#)

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